

Paediatric Posterior Fossa Surgery Post-Operative Guideline

Assess pain scores (FLACC or numerical) at rest and on neck movement

Assess for level of sedation and any side-effects

DAY ZERO

REGULAR

PARACETAMOL: PO or IV

OMEPRAZOLE: 10mg (10–19 kg) or
20mg (20kg and over) - whilst on steroids

ONDANSETRON: 0.1mg/kg IV TDS

MORPHINE:

If < 30kg or unable to use a PCA →

MORPHINE INFUSION 5-40mcg/kg/hr

If >30KG and able to use a PCA →

MORPHINE PCA +- BACKGROUND

Background of 10-20mcg/kg/hr may be appropriate for 1st post-operative night whilst the child is drowsy and not pressing the button.

PRN

CYCLIZINE: slow IV TDS

NALOXONE: IV 100mcg/kg (max 2mg)

DAY ONE

Day Zero meds plus:-

Can start without consultation with the neurosurgeon if child >12 hrs post-op, no complications and neurologically appropriate

IBUPROFEN: Max 30mg/kg in 24 hrs
OR **DICLOFENAC:** PO/PR 1mg/kg TDS (max single dose 50mg)

DIAZEPAM: for neck spasm, start at lower dose but can increase if child not overly sedated

1-4 yrs 1.25 - 2.5mg
5-11yrs 2.5 - 5mg
12 and over 5 - 10mg

MOVICOL: PO 1 sachet BD

OR **DOCUSATE SODIUM:**

2–11 yrs - 12.5–25 mg tds
12-17yrs – 100mg tds

DAY TWO – FOUR

When ready to stop IV morphine:-

Tolerating fluids with no vomiting

Pain scores ≤4/10 on movement as well as rest

Established on oral paracetamol/ibuprofen

Rescue analgesia prescribed

DIHYDROCODEINE:

Regular or prn depending on pain

1mg/kg max 30mg (0.5mg/kg in children < 4)

OROMORPH:

200mcg/kg 4 hourly prn

Consider 2nd **LAXATIVE** if not opened bowels by day 3

* In some older children it may be appropriate to skip the IVI morphine approach and proceed directly to dihydrocodeine/oromorph.

IV morphine rescue boluses can be delivered in PICU but not on Nicholls. *

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